

MEDCORE HOSPITALS

Patient Registration Policy

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Document Control

This document is maintained as an internal operating standard for MedCore Hospitals. The current version, effective date, owner and classification are shown on the document-control panel. Staff should use the current approved version and should not rely on locally saved copies when a newer controlled version has been issued. Material changes to registration workflows, systems, responsibilities or escalation routes should trigger a review of this document.

1. Purpose

The purpose of this policy is to establish a consistent approach to patient registration and maintenance of administrative identity information. Accurate registration supports appointment scheduling, billing, communication and retrieval of the correct patient information. The policy emphasizes identity accuracy, search-before-create discipline, traceability, appropriate handling of exceptions and controlled access to registration information.

2. Scope

The policy applies to reception, Patient Services, registration counters, Records staff and other authorized administrative employees who create, verify, update or use patient registration information. It covers new patients, returning patients, emergency presentations, unidentified patients, duplicate records, demographic updates, payer information, communication preferences and downtime registration. It also establishes escalation expectations when normal processing cannot be completed.

3. Definitions

A patient record is the administrative identity used to associate a person with hospital services. A duplicate is a second record that appears to represent the same person as another existing record. A temporary identity is an approved record used when reliable identity information is not immediately available. A material correction is a change that can affect identity, downstream processing, communications or financial administration and therefore requires an authorized workflow.

4. Registration Principles

Registration staff should prioritize identity accuracy and traceability. Search should occur before creation, information should be obtained from an appropriate source, mandatory fields should not be populated with guessed values, and uncertainty should be visible rather than hidden. Staff should work within their authorization and should escalate matters that require Records, Finance, Legal & Compliance, IT Support or another responsible function.

Control	Standard practice	Responsible role	Escalation trigger
Routine	Follow the approved workflow and document material actions.	Process owner	Exception cannot be resolved.
Verification	Use authorized information and do not guess.	Assigned staff	Identity or source is uncertain.
Traceability	Keep status, owner and next step visible.	Department manager	Case becomes overdue or conflicting.

5. Search-Before-Create Procedure

The registration process begins by searching for an existing patient identity using available identifiers. Staff should review plausible matches rather than selecting the first similar name. The search should consider combinations of identifiers available in the registration interface. If one record is clearly supported, the existing identity should be used. If multiple records remain plausible, the employee should stop the create process and route the case for authorized review.

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6. New Patient Registration

When no appropriate existing record is identified, staff may proceed with new registration. The employee should capture required demographic and contact information, enter applicable payer information, record communication preferences where supported and review the completed fields before finalizing. The employee should check for obvious spelling or transcription errors and confirm that the record has been attached to the intended encounter or service workflow.

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7. Returning Patient Registration

Returning patients should normally be associated with their existing record. Staff should verify identity and confirm information that may have changed. Address, telephone, email, payer information and communication preference changes should be handled as updates. Staff should not create a new record merely because a field is missing or outdated. If identity cannot be confirmed, the case should be escalated before proceeding.

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8. Emergency Registration

Emergency presentations can require administrative processing before complete identity information is available. The approved temporary or emergency registration process should be used. Only information required for immediate processing should be recorded when identity is uncertain. Once reliable information becomes available, authorized staff should reconcile the temporary identity with the correct patient record and preserve the reconciliation trail.

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9. Unidentified Patients

When a patient cannot reliably provide identifying information, staff should use the approved unidentified-patient process rather than inventing demographic data. The temporary identity should be clearly managed so that later information can be reconciled. Any unresolved identity conflict should be escalated to Records. Staff should avoid creating multiple temporary identities for the same presentation when the approved workflow provides a mechanism to maintain one controlled record.

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10. Demographic Data Capture

Demographic information should be entered accurately from an appropriate source or patient confirmation. Staff should review names, dates and contact details for transcription errors. If a required field cannot be obtained, the approved exception method should be used. Information belonging to another person should not be copied into a patient record without an authorized relationship or data-sharing process.

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11. Contact Information and Preferences

Contact information should reflect the patient's current approved contact details. Staff should confirm telephone numbers, email addresses and other relevant fields when the workflow requires it. Communication preferences should be recorded using the designated fields. Where a patient requests a communication method that is not available through the standard process, staff should route the request to the responsible function instead of making an unsupported commitment.

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12. Payer and Insurance Information

Where payer information is captured, staff should use approved documentation or patient-provided information and follow the applicable verification process. Administrative staff should not guarantee coverage, reimbursement or claim outcomes. Questions about eligibility, financial responsibility or disputed charges should be routed to Finance or the Insurance function. Payer changes should update the appropriate existing record rather than trigger creation of another patient identity.

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13. Duplicate Records

A suspected duplicate should be flagged and reviewed by authorized Records staff. Review should compare the available identifiers and determine whether the records represent the same individual. Staff should not merge, delete or materially alter identities outside their authorization. Duplicate patterns should be monitored because repeated duplicates may indicate unclear search practices, data-entry issues or workflow gaps.

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14. Corrections and Amendments

Corrections should use the approved correction workflow and remain traceable. Staff should distinguish routine updates from material corrections. A correction affecting identity, payer information, communication or downstream processing may require review by an authorized function. The original issue should not be hidden through informal overwriting when the system provides a controlled amendment or audit mechanism.

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15. Confidentiality and Access

Registration information should be accessed only for legitimate work requirements. Staff should avoid discussing sensitive information where unauthorized persons can overhear. Identity should be verified before sensitive administrative information is disclosed. Screens, printed material and conversations should be handled in accordance with approved confidentiality practices. Privacy concerns should be escalated through the designated route.

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16. Downtime Registration

When the registration system is unavailable, staff should use the approved downtime process and capture essential information needed for immediate operations. Offline information should be protected from unauthorized access. After restoration, staff should reconcile temporary registrations and updates into the system, verify that duplicates have not been introduced and close outstanding downtime exceptions.

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17. Roles and Responsibilities

Registration staff own routine search, verification and data capture. Records owns identity conflicts, duplicate review and authorized reconciliation. Patient Services coordinates patient-facing exceptions. Finance or Insurance owns payer-related questions. IT Support owns system availability issues. Department managers are responsible for recurring quality problems, overdue escalations and ensuring that staff understand the current workflow.

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18. Escalation and Exception Handling

Escalation is required when the employee cannot establish the correct identity, when multiple plausible records exist, when a material correction is requested, when privacy is questioned, when payer information is disputed or when downtime leaves unresolved records. An escalation should include the relevant patient or encounter reference, the issue identified, actions already taken and the next decision required. This prevents repeated work and incomplete handoffs.

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19. Quality Controls and Audit

Management may review duplicate rates, registration completeness, correction volumes, temporary-record reconciliation, unresolved exceptions and recurring registration errors. Quality review should examine whether staff followed the process rather than focusing only on outcomes. Sampling can verify that search-before-create was performed, identity was checked and material changes used controlled workflows.

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20. Operational Scenarios

Scenario A: a returning patient has a changed address; update the existing record. Scenario B: two records have similar identifiers; stop and route to Records. Scenario C: an emergency patient lacks reliable identity information; use the approved temporary process. Scenario D: a caller requests sensitive information but cannot complete verification; provide only information permitted without verification and route appropriately. Scenario E: the system is unavailable; use downtime processing and reconcile after restoration.

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21. Frequently Asked Questions

Staff should not skip the search merely because the patient appears new. Missing information should not be guessed. A suspected duplicate should be routed to Records. A clinical question should not be answered by administrative registration staff. A payer dispute belongs with the appropriate Finance or Insurance function. If a workflow is unavailable, staff should use the approved downtime process rather than creating uncontrolled workarounds.

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22. Governance and Review

The document owner should review this policy periodically and whenever there is a material change to the registration system, patient identity process, departmental responsibility or escalation structure. Updates should be communicated to affected staff. Obsolete copies should not remain in active operational use. Feedback from recurring exceptions and audit findings should be considered during review.